



Laboratory Investigation Report

Patient Name	: Sonam	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 28 Y 0 M 28 D /F	OP/IP No	: OP/101318
Max ID/Mobile	: 435028/8752005200	Collection Date/Time	: 10/Sep/2021 02:16PM
Lab ID	: 1266092100236	Receiving Date	: 11/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
Passport No.	:		

Blood Bank

Blood Grouping and RH Factor*, EDTA

Date	10/Sep/2021 02:16PM	Unit	Bio Ref Interval
Blood Group	AB		
Haemagglutination	POSITIVE		

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Sangeeta Pathak , DIHBT
Head-Transfusion Med



SIN No: B2B899475, Test Performed at : 1103 - Max Hospital Saket (East Block), 1, 2, Press Enclave Marg, Saket Institutional Area, Saket, New Delhi
Max Lab - A Division of Max Healthcare Institute Ltd.

Booked at: Max Super Specialty Hospital, Saket (West Block), 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

8752005200
CIN No.: L72200MH2001PLC322854
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Clinical Biochemistry

Blood Sugar (Random), Fluoride Plasma*

Date	10/Sep/2021 02:20PM	Unit	Bio Ref Interval
Random Glucose UW-Hexokinase	79.84	mg/dl	74 - 140

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***

Mrinalini
Dr. Mrinalini Priyadarshini
 DCP, DNB (Path.)
 Consultant Pathologist



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Clinical Pathology

Urine Routine And Microscopy*

Date	10/Sep/2021 02:22PM	Unit	Bio Ref Interval
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Macroscopy

Reflectance photometry

Colour	Pale Yellow		Pale Yellow
PH	6.5	..	5-6
Specific Gravity	1.025		1.015 - 1.025
Protein	Nil		Nil
Glucose	Nil		Nil
Ketones	Nil		Nil
Blood	Nil		Nil
Bilirubin	Nil		Nil
Urobilinogen	Normal		Normal
Nitrite	Negative		

Microscopy

Light Microscopy/Image capture microscopy

Red Blood Cells (RBC)	Nil	/HPF	Nil
White Blood Cells	2-4	/HPF	0.0-5.0
Squamous Epithelial Cells	1-2	/HPF	
Cast	Nil	/LPF	Nil
Crystals	Nil	..	Nil
Bacteria	Nil	/HPF	Nil

Kindly correlate with clinical findings

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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma*, Sodium Citrate

Date	10/Sep/2021 02:18PM	Unit	Bio Ref Interval
APTT	30.1	Sec	
Photo-Optical-Nephelometry			
Control	28.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the “intrinsic coagulation” pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - ‘APTT mixing study’, ‘Lupus Anticoagulant test’, ‘specific Factor(s) assay’ may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma*

Date	10/Sep/2021 02:18PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	11.4	Sec	12.1-15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	1.00		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the “extrinsic coagulation” pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - ‘PT mixing study’, ‘specific factor(s) assay’ may be added on for further evaluation.



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Hematology

CBC (Complete Blood Count)*, Whole Blood EDTA

Date	10/Sep/2021 02:24PM	14/Aug/21 08:08AM	Unit	Bio Ref Interval
Haemoglobin	16.41	15.04	g/dl	12.0 - 15.0
Packed Cell, Volume Calculated	50.8	44.5	%	36-46
Total Leucocyte Count (TLC) Electrical Impedance	6.13	7.18	10~9/L	4.0-10.0
RBC Count Electrical Impedance	5.93	5.32	10~12/L	3.8-4.8
MCV Electrical Impedance	85.6	83.6	fL	83-101
MCH Calculated	27.7	28.3	pg	27-32
MCHC Calculated	32.3	33.8	g/dl	31.5-34.5
Platelet Count Electrical Impedance	251.1	193.4	10~9/L	150-410
MPV Calculated	11.09	10.51	fl	7.8-11.2
RDW Calculated	13.1	13.5	%	11.5-14.5

Differential Cell Count

VCS / Light Microscopy

Neutrophils	62	65	%	40-80
Lymphocytes	25	24	%	20-40
Monocytes	07	09	%	2-10
Eosinophils	06	01	%	1-6
Basophils	00	01	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	3.8	4.67	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.5	1.7	10~9/L	1.0-3.0
Absolute Monocyte Count	0.43	0.65	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.37	0.07	10~9/L	0.02-0.5



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Hematology

Kindly correlate with clinical findings

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Immunoassay

FSH - Follicle Stimulating Hormone,Serum*

Date	10/Sep/2021 02:20PM	Unit	Bio Ref Interval
Follicle Stimulating Hormone	7.92	mIU/mL	
CLIA			

Ref. Range

Adult Male	1.27 - 19.26
Adult Female :	
Follicular	3.85 - 8.78
Midcycle Peak	4.54 - 22.51
Luteal Phase	1.79 - 5.12
Post Menopausal (>50 Yrs)	16.74 - 113.59

Interpretation

Increased in primary gonadal failure, ovarian or testicular agenesis, Klinefelter's syndrome, Reifenstein's syndrome, castration, alcoholism, menopause, orchitis, gonadotropin-secreting pituitary tumors.

Decreased in anterior hypofunction, hypothalamic disorders, pregnancy, anorexia nervosa, polycystic ovarian disease, hemochromatosis, sickle cell anaemia, severe illness, hyperprolactinemia.

Pooled samples are advisable due to episodic, diurnal and cyclic variations in gonadotropin secretion.



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Immunoassay

LH-Luteinizing Hormone , Serum*

Date	10/Sep/2021 02:20PM	Unit	Bio Ref Interval
Luteinizing Hormone CLIA	6.61	mIU/mL	

Ref Range

LH(Male-Adult)	Reference Range
	1.24-8.62
LH (Female-Adult)	
Follicular	2.12-10.89
Mid Cycle Peak	19.18-103.03
Luteal Phase	1.2-12.86
Post Menopausal (>50 Year)	10.87-58.64

Interpretation

Increased in Primary gonadal dysfunction, polycystic ovarian syndrome (LH/FSH ratio is high in 60% cases), post-menopause, and pituitary adenoma. Decreased in pituitary or hypothalamic impairment, isolated gonadotropic deficiency associated with anosmia or hyposmia (Kallmann's syndrome), anorexia nervosa, isolated LH deficiency ("fertile eunuch"), sever stress, malnutrition, and sever illness. Pooled samples are advisable due to episodic, diurnal and cyclic variations in gonadotropin secretion.



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Immunoassay

Prolactin, Serum*

Date	10/Sep/2021 02:20PM	Unit	Bio Ref Interval
Prolactin	14.65	ng/mL	
CLIA			

Ref Range

Males :	2.64 - 13.13
Females :	
Premenopausal	
(<50 years of age):	3.34 - 26.74
Postmenopausal	
(>50 years of age):	2.74 - 19.64

Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery

Decreased in pituitary apoplexy (Sheehan's Syndrome)



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Immunoassay

TSH,Serum*

Date	10/Sep/2021 02:22PM	Unit	Bio Ref Interval
Thyroid Stimulating Hormone	1.7	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

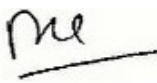
Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



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Patient Name	: Sonam	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 28 Y 0 M 28 D /F	OP/IP No	: OP/101318
Max ID/Mobile	: 435028/8752005200	Collection Date/Time	: 10/Sep/2021 02:22PM
Lab ID	: 1266092100236	Receiving Date	: 10/Sep/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 11/Sep/2021
Passport No.	:		

Immunoassay

Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



SIN No: B2B899475, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

Boo Max Lab - A Division of Max Healthcare Institute Ltd. Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752 Max Super Specialty Hospital, Saket (West Block)

1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.
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CIN No.: L72200MH2001PLC322854

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Serology

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test Negative

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR

A non reactive result does not exclude the possibility of exposure to or infection with HCV.

Patients with auto immune liver disease may show falsely reactive results





Laboratory Investigation Report

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Test Name	Serology Result	Unit	Bio Ref Interval
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VDRL/RPR*, Serum

RPR(Syphilis) Non Reactive
Slide Flocculation

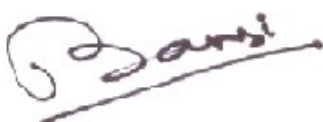
Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

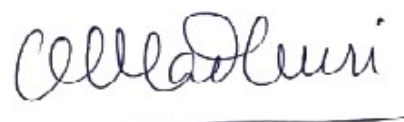
*** End Of Report ***



Dr. Bansidhar Tarai, M.D.
Associate Director
Microbiology & Molecular Diagnostics



Dr. Poornima Sen, M.D.
Consultant - Microbiology



Dr. Madhuri Somani, M.D. , DNB
Consultant - Microbiology



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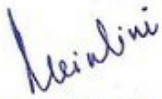


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Serology

Test Name	Result	Unit	Bio Ref Interval
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DCP, DNB (Path.)
Consultant Pathologist



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Immunoassay

Estradiol (E2),Serum

Date	10/Sep/2021 02:22PM	Unit	Bio Ref Interval
Estradiol CLIA	33	pg/mL	

Ref Range

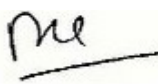
Male	20 - 75
Post - Menopausal (Female)	20 - 88
Estradiol -Total (Non - Pregnant Females)	
Mid Follicular Phase	24 - 114
Mid - Luteal Phase	80 - 273
Periovulatory	62 - 534

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



SIN No:B2B899475, Test Performed at :910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017
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Test Name	Serology	Result	Unit	Bio Ref Interval
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HIV Test (I and II),Serum

CLIA

HIV (I and II) Negative

Interpretation

- 1) A Negative result implies that no Anti HIV-1 or HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been taken during the "WINDOW PERIOD" (before the development of detectable levels of antibodies).
- 2) Positive result suggests the possibility of HIV-1 or HIV-2 infection.

Advise:

To rule out false positivity, false negativity and window period, kindly perform "Confirmatory Tests" like HIV I RNA (Qualitative) Real Time PCR.

Hepatitis B Surface Antigen,Serum

CLIA

HBsAg Test Value 0.22

Ref. Range

Negative	< 0.90
Borderline	0.90 - 5.0
Positive	> 5.0

Interpretation

- This test is used to detect hepatitis B surface antigen (HBsAg) in serum sample based on VITROS immunometric immunoassay technique and aid in the laboratory diagnosis of HBV infection.
- Viral hepatitis is a major public health problem with an estimated 257 million persistent carriers of hepatitis B virus (HBV) worldwide. Infection with HBV results in a wide spectrum of acute and chronic liver diseases that may lead to cirrhosis and hepatocellular carcinoma.
- Transmission of HBV occurs by percutaneous exposure to blood products, needle stick injury, sexual contact and perinatally from HBV-infected mothers to baby.
- Hepatitis B surface antigen (HBsAg), derived from the viral envelope, is the first antigen to appear following infection.
- Positive results should be correlated with other potential laboratory abnormalities and clinical picture.
- A negative test result does not exclude the possibility of exposure to or infection with hepatitis B virus.
- Levels of HBsAg may be undetectable both in early infection and late after infection.



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- In rare cases HBsAg tests do not detect certain HBV mutant strains.
- HBs Ag disappears with recovery from clinical disease in most patients, however, it persists for years in carriers.

Kindly correlate with clinical findings

*** End Of Report ***



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